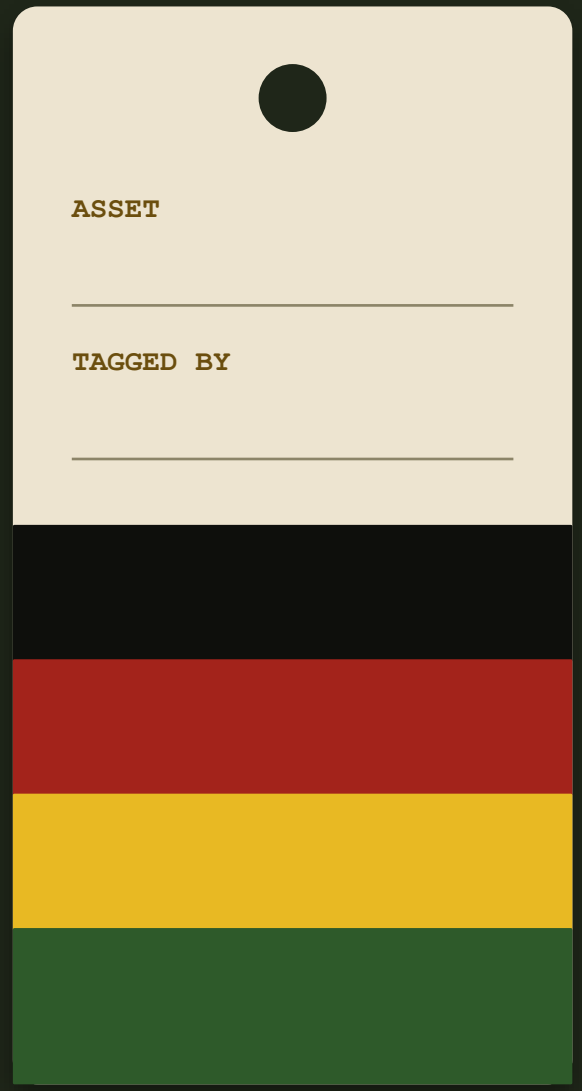
TRIANGLE INFOSECON · 30 OCT 2026

Triage Before Treatment

Running incident tabletops like a trauma bay

Steven Lorenz



One voice calls it.

Everyone else is quiet, and working.

WHO'S TALKING

Doc Actual

THEN

Hospital Corpsman, FMF

2d LAR

WCN Naval Hospital Pensacola

NOW

Chief Security Architect

Helping SMBs find a safe path forward of the security poverty line.

Nobody thinks clearly at the worst moment.

So the format thinks for you.

Most tabletops test what your team knows.

Your worst day tests what your team does.

A trauma bay rehearses doing. So can a tabletop?

> LET ME SAY IT AGAIN. TRAIN FOR THE WORST. MUSCLE MEMORY WINS THE DAY.

Most tabletops are a quiz with snacks

01

THEY TEST RECALL

Where's the IR plan? Who's the insurance carrier? Nobody is ever short on time or hands during a TTX.

02

NOBODY LEADS

Everyone talks to the room. Often at the same time. Decisions float until someone gets tired.

03

THE GAP LIST

Twenty findings, no owners. Next year, same list, better font, maybe? Same issued? Probably!

Who in your organization can stop everything?

Freeze the change window. Pull the CFO off a call. Declare the incident.

Not who should. Who can.

Two ideas. That’s the whole method.

ONE

Order

One commander. Everyone reports to them.

Find what’s killing it before you fix anything.

Triage before treatment.

TWO

Clear updates

Going up: a structured escalation, not a ramble.

Going across: a handoff that says what you changed.

Same idea both ways. Nothing forgotten.

Every tool already works under pressure

ORDER

ICS

Incident Command System

One commander. Everyone reports up.

Fire service

ORDER

START

Mass-casualty triage

Sort before you treat. Four colors.

EMS and fire

GOING UP

SBAR

Situation, Background, Assessment, Recommendation

How you escalate to the person who decides.

Navy submarines, then hospitals

GOING ACROSS

MIST

Mechanism, Injuries, Signs, Treatment

What you hand the next team.

Military and EMS trauma handoff

Tag before you treat

EXPECTANT	Accept the loss. Move on.
IMMEDIATE	Operations stopped
DELAYED	Degraded, still running
MINOR	Walking wounded/minimal intervention

THE HARDEST TAG

Who is allowed to call a system expectant?

If nobody can say “we’re letting this one go,” everything stays red forever.

EXPECTANT — Not a failure. A decision.

SBAR

Situation, Background, Assessment, Recommendation. Thirty seconds, in order.

S	SITUATION	What's happening, in one sentence
B	BACKGROUND	What's affected, counted by triage color
A	ASSESSMENT	What you think it is. Start hostile. Prove your way down.
R	RECOMMENDATION	What you need from them, and by when

The handoff

M

MECHANISM

How it got in, as far as you know

I

INJURIES

What's down, damaged, or exposed

S

SIGNS

What you're seeing right now

T

TREATMENT GIVEN

Everything you already changed

T is the one most tech teams can't answer. Every reset, reimage, and block changed the evidence.

Let’s run one anyway

ME

I run the clock.

I read the injects.

I ask the questions nobody wants asked.

YOU

You’re the IR team. Answer out loud, or call out the color.

Kestrel Manufacturing

300	1 + 1	2	06:40
seats	plant, office	admins on shift	Friday

Nobody planned for it to be another Monday. Nobody ever does.

INJECT 1

06:40

The plant floor can’t reach the ERP. File shares are full of renamed files.

DECIDE Who’s leading? Say it out loud.

06:55

Five systems are down. You have two admins on shift.

- ERP
- File server
- Email
- Badge access
- CEO's laptop

DECIDE Tag them. What has to wait?

07:10

The CEO calls. “What’s going on?”

DECIDE Give the CEO an SBAR. Thirty seconds.

07:25

A service account just logged on to a domain controller. Also: a legitimate admin already reset two passwords and reimaged a server… {I call this “helping”}.

DECIDE What’s your assessment now? And what did we just lose?

08:00

Your retainer firm arrives. First question: what have you changed? Actually, that is probably the second thing. The first thing they asked for was network and dataflow diagrams and all you have were updated in 2017, if you’re lucky.

DECIDE Hand it over. Mechanism, injuries, signs, treatment given.

08:30

The same service account logged in to the backup console last Thursday.

DECIDE Is the way back clean? Clean, suspect, or unknown?

Hot wash

WHAT WORKED

Three things.

WHAT DIDN'T

Three things. Be honest

WHO OWNS IT

A name on every fix.

What we just did

INJECT 1	**A leader, named before the clock starts**	Order has to start with someone.
INJECT 2	**More red tags than hands**	Scarcity forces choice.
INJECT 3	**Every escalation is an SBAR**	The update is the thing being rehearsed.
INJECT 5	**Someone new arrives mid-crisis**	And asks what you changed.
HOT WASH	**A debrief while it's fresh**	Every finding leaves with an owner.

The tabletop kit

The Kestrel scenario and all six injects, facilitator notes, an SBAR guide, a triage sheet, a MIST handoff sheet, and a hot wash template.

Try it for your next tabletop.

Seriously.



https://stevenlorenz.dev/triage

BACK TO KESTREL

Assessment: Backups unknown.

We don’t know if the way back is clean. Find that out in a tabletop, not on your worst day.

Say it in order.

Every time.

Three things. No budget. No permission.

01

Name your commander

Before you need one. Write it down where people can find it at 06:40.

02

Make every escalation an SBAR

Situation, background, assessment, and what you need from them.

03

Write down every change

Every reset, reimage, and block. That list is the T in MIST.

Pick one.

Try it in your next tabletop. See if it holds.

Not all of it at once. Not a roadmap with a Gantt chart.

One thing, a little better, one minor course correction at a time.

Questions